

إسم المريض ربيعاً : زينب محمد حسين حسين
المحافظة : القاهرة الحي / المدينة : الاميري
رقم البطاقة : 26903310101867 رقم التليفون : 01123704042
العمر(تاريخ الميلاد) : 31-03-1969 المهنة : بدون عمل
العنوان : ش على السعداوى
تاريخ تسجيل الإستمارة : 05-10-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

GRI-٤

رستم خیل

مدرسہ اسلامیہ

صحة الامانيات المرفعة

للجنة ومدير

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: زينب محمد حسين حسين National ID: 26903310101867	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



زینب
محمد حسين حسين
١٦ شارع علي السعداوي
الاميرية - القاهرة

٢٦٩.٣٣١.٠١.١٨٦٧

JA4054909

٢٠٢١/٠٨ ٢٦٩.٣٣١.٠١.١٨٦٧

ربة منزل
أنثى
مسلمة

البطاقة سارية حتى ٢٠٢٨/٠٨/٢٦



Patient Name : د. أسماء ماجدي / Dr. Asmaa Magdy
Patient No. : 26903310101867
Path No. : 2304/24
Sex : Female
Age : 55 Y
Referring Doc :

Request No. : 13001460
Request Date : 08/07/2024
Verification Date : 15/07/2024
Financial Category : Out Patient
Department :
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data:

Right breast cancer post NACT.

Nature of the specimen:

Right Simple mastectomy and sentinel lymph node biopsy specimens, collected at 8/7/2024.

Gross Examination:

Two specimens were received:

- Sentinel lymph node biopsy specimen: Fibrofatty tissue piece measured 4x3 cm enclosing 5 nodules measured 1.5x1 cm.
- Simple mastectomy specimen measured 24x15x5 cm with covering skin ellipse measured 18x10 cm.

• Tumor extent

- o Number: An ill defined greyish white firm mass.
- o Size: 3.3x2.5x1 cm.
- Distance from margins: 2 cm from deep margin.
- Additional findings: There was a grey white rubbery area glistening measured 5x4 cm.
- Frozen section:

- Sentinel lymph nodes: Positive (1/5).

• Gross key:

- 1, 2, 3- Mass
- 4- Rubbery grey white area.
- 5- Nipple and areola
- 6- Deep margin.
- 7- Sentinel nodes.

Microscopic examination:

- Histological type: Invasive duct carcinoma.

Verified by:

Dr. Asmaa Magdy

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Name: زينب محمد حسين حسين			
MR. : 5820			
Detailed Medical Report			
		السيد	
تحيّة طيبة وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		بقسم:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient presented for initiation of Femara therapy, with supportive treatment planned for three months; the status of the supportive therapy was pending at the time of the visit. The patient was scheduled for a dental examination prior to starting Zometa, and was instructed to bring the recent SM report and blood sugar report for review. The patient has a history of right breast cancer treated with a simple mastectomy on 8/7/2024. Pathology was T2 N1/5, IDC grade 2, ER positive, PR positive, HER2 negative, Ki-67 10%. The patient received adjuvant chemotherapy with Taxol and AC, followed by a 1-year course of Femara starting in January 2025, and is now scheduled to begin Zometa.			
تحريراً في ,			
التاريخ :		20-09-2026	
الوقت :		dina makram	
الطبيب المعالج :			
يعتمد ,			

Patient Name: Zahrah Mohamed Hussein
Hussein

Date: 21/05/2024

Exam: C.T CHEST

Age: 54 Year(s)

Referral ID: 1550/2024

Pt ID: 54045

NON-CONTRAST PULMONARY CT SCAN OF THE
CHEST ABDOMEN AND PELVIS
XRAY

Findings:

Right breast: lobulated mass lesion measures about 2.7x2 cm.
multiple prominent right axillary level 1 lymph nodes, largest
1.3x1.2mm.

Multiple prominent level 1 left axillary lymph nodes, largest
0.8x0.6mm.

bilobed middle lobe subpleural bands

Clear cost of both lung fields. No sizable pulmonary nodules,
masses, areas of consolidation or cavitary lesions.

No hilar or mediastinal lymph nodes.

No evidence of pleural effusion, pleural thickening or masses
seen.

Normal cardiac size and shape with no gross abnormality of the
cardiac chambers, no pericardic effusion detected.

Left thyroid lobe calcific focus.

bilobed elastofibrous densi.

Abdominal findings:

The liver is of average size with homogeneous CT texture. No
focal lesions or contour irregularities.

No intra-hepatic or extra-hepatic biliary ducts dilatation

Normal size of pancreas, normal texture of the spleen with no focal
lesions seen. Small splenic splenule is seen.

Small sized right kidney with irregular outlines and focal areas
of parenchymal thinning. Right upper calyceal tiny stones.

2mm, 2.15mm, 2mm and 2mm hydrocalyxes.

Normal sized, prominent and the loss of density of left kidney with
evidence of renal pyramids enlargement.

Patient Name : **Esraa Magdy**
Patient No. : **20903310101807**
Path No. : **2304/24**
Sex : **Female**
Age : **55 Y**
Referring Doc :

Request No. : **13001400**
Request Date : **08/07/2024**
Verification Date : **15/07/2024**
Financial Category : **Self Pay**
Department : **Out Patient**
Registered by : **Mrs. Mayada Hegazy**

Pathology Report

- Grade (differentiation + nuclear pleomorphism + mitosis) : **G II (3+2+1)**
- Actual tumor cellularity: **65%**
- DCIS: **Not detected**
- Angiolymphatic invasion: **Occasional**
- Skin involvement: **Free nipple and areola**
- Lymph node involvement: **Positive (1/5)**
- Resection Margins: **Free deep margin**
- Additional pathologic findings: **Fibrocystic changes and apocrine metaplasia in rubbery greyish white area**
- Histologic response to therapy: **Pathologic partial response**

Diagnosis:

Right breast cancer post NACT, simple mastectomy and sentinel lymph node biopsy specimens:

- Invasive duct carcinoma, grade II.
- Free nipple and areola.
- Free deep margin.
- Lymph node involvement: **Positive (1/5)**
- Pathologic partial response.
- Stage : **ypT2, ypN1**
- RCB: **3.782, class: III**

Prof. Gamal Abdelatty
Dr. Menno Nabil

Verified by :
Dr. Esraa Magdy

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رقم الغرفة (4) - الطابق الأول - مبنى رقم 987 - شاطئ النيل - مصر القديمة - القاهرة، مصر

Patient Name : الأمانة محمد حسن / Amani Mohamed Hassan
Patient No. : 26903310101867
Path No. : SR-647/24
Sex : Female
Age : 55 Y
Referring Doc. :

Request No. : 13000387
Request Date : 22/05/2024
Verification Date : 02/06/2024
Financial Category : أورام ناز - أورام - أورام - أورام
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.

Nature of Specimen: Slide & block revision

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

IPT (Positive control showed appropriate reactivity):
ER: Moderate nuclear reaction in 90% of tumor cells (score 7/8).
PR: Moderate nuclear reaction in 80% tumor cells (score 7/8).
Her2: No membrane reaction in tumor cells (score 0).
Ki67: 10% nuclear reaction of tumor cells.

Conclusion:

Right breast cancer, Slide & block revision:
Invasive duct carcinoma, Grade II, with minimal DCIS.

Immunohistochemistry:

ER: Positive (score 7/8).
PR: Positive (score 7/8).
Her2: Negative (score 0).
Ki67: Positive in 10% of tumor cells.

Scoring system: "ER, PR"

SCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by:

Dr. Hoba Hasan

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Patient Name: الأمانة / زوزن محمد حسين
Patient No.: 26903310101867
Path No.: SR-647/24
Sex: Female
Age: 55 Y
Referring Doc:

Request No.: 13000387
Request Date: 22/05/2024
Verification Date: 02/06/2024
Financial Category: اورشيد - فئة قسرة
Department: Out Patient
Registered by: manwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, Intense < 10% of tumor cells.

Signature
Dr. Rana Fathy
Dr. Heba Hassan

Verified by:
Dr. Heba Hassan

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